This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Hepatitis A Vaccine (Havrix/Avaxim), Typhoid Vaccine (Typhim Vi), Cholera Vaccine (Dukoral) for the prevention of travel-related infection

Patient Group Direction, version 008, issued 30 September 2026. Supersedes Version 007, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for travel vaccination: Green Book chapters 17 (Hepatitis A), 33 (Typhoid) and 14 (Cholera), NaTHNaC / TravelHealthPro country information and the SmPCs of the vaccines administered. NICE CKS 'Immunizations - travel' summarises the same sources.

Pre-Travel Health Assessment

Pre-travel health assessment is essential for all travelers to identify and manage health risks associated with their destination, duration of travel, planned activities, and individual medical factors.

Risk Assessment

- **Destination and Duration:** Assess the epidemiological risks in the specific country or region, including current outbreaks and seasonal variations. Duration of stay and type of accommodation influence risk.
- **Activities:** Planned activities (trekking, water activities, visiting rural areas) may increase exposure to vaccine-preventable diseases.
- **Medical History:** Previous vaccinations, immunological status, chronic medical conditions, and medication use must be considered.

Assessment

- **Travel Itinerary Review:** Confirm specific countries/regions, duration, accommodation type, and planned activities.
- **Vaccination History:** Review existing immunity, previous travel vaccinations, and childhood immunisations.
- **Current Medications:** Review for interactions with vaccines and assess if immunosuppressed.
- **Medical Conditions:** Assess for contraindications, special precautions, or need for specialist advice.

Management

- **Vaccination Schedule:** Provide appropriate vaccinations according to travel risk, allowing adequate time between doses where required.
- **Preventive Measures:** Advise on food/water safety, insect bite avoidance, personal hygiene, and malaria prophylaxis if indicated.
- **Specialist Advice:** Consider referral for high-risk travelers or complex medical circumstances.

Red Flags and Referral Criteria

- **Immunocompromised Patients:** Seek specialist advice for HIV+, on immunosuppressants, or with primary immunodeficiency.
- **Pregnant Travelers:** Avoid live vaccines; discuss safe travel timing and other preventive measures.
- **Infants (<6 months):** Most vaccines contraindicated; refer for specialist advice.
- **Severe Allergies:** Consider alternative destinations or specialist pre-travel assessment.

Patient Group Direction

for the supply/administration of

Hepatitis A Vaccine (Havrix/Avaxim)

for protection against

Hepatitis A (travel)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first

	working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Protection against Hepatitis A for travelers to endemic areas
Inclusion criteria	Adults aged 18 years and over Traveling to areas with high or intermediate prevalence of Hepatitis A No previous complete Hepatitis A vaccination course No documented evidence of Hepatitis A immunity
Exclusion criteria	Known hypersensitivity to vaccine or any excipients Acute illness with fever (defer until recovered) Pregnant women (seek specialist advice)
Cautions	Thrombocytopenia, a bleeding disorder or anticoagulation: not a contraindication. Use a fine needle and apply firm pressure for 2 minutes; Havrix may be given deep subcutaneous where local guidance requires. Caution in patients taking anticoagulant therapy Caution in immunocompromised patients (may have reduced response; seek specialist advice)

	Give intramuscularly; do not administer intravenously or intradermally Vaccinate at least 2 weeks before departure if possible
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Havrix Monodose 1440 EL.U/1.0 mL, or Avaxim 160 U/0.5 mL: inactivated hepatitis A vaccine injection
Legal category	POM
Storage	Store at 2-8°C. Do not freeze. Protect from light. Do not use if frozen.
Route/method of administration	Intramuscular injection in the deltoid region
Dose and frequency	Primary course: one dose. Havrix: 1.0 mL. Avaxim: 0.5 mL. Booster dose: Required at 6-12 months after primary dose for long-term protection (10+ years)
Quantity to be administered and/or supplied	One dose per visit (1.0 mL Havrix, or 0.5 mL Avaxim)
Maximum or minimum treatment period	Single dose for protection; booster dose required at 6-12 months
Adverse effects	Common (>1/100): Injection site reactions (pain, redness, swelling), headache, fatigue Uncommon (>1/1000): Myalgia, fever, nausea, abdominal pain Rare: Allergic reactions including anaphylaxis Very rare: Guillain-Barré syndrome, convulsions

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of administration, dose, form, route and anatomical site,

	quantity administered, batch number and expiry date • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and the actions taken, and that the vaccine was administered under this PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each vaccine. Ensure patient understands the importance of completing the full course if required and understands booster schedules.
Follow-up advice to be given to patient or carer	Complete the full course recommended for the vaccine given: cholera, 2 doses at least 1 week apart, finished at least 1 week before exposure; hepatitis A, a second dose at 6 to 12 months for long-term protection. Attend for booster vaccinations as advised (Hepatitis A at 6-12 months; Typhoid every 3 years; Cholera every 2 years). Report any serious side effects to the GP or seek medical attention immediately. Maintain good food and water hygiene practices during travel (handwashing, avoid raw/undercooked foods, avoid tap water in endemic areas). Obtain comprehensive travel health insurance that covers medical evacuation if traveling to remote areas. Visit your GP or travel health clinic at least 4-6 weeks before departure for full pre-travel assessment. Report symptoms of Hepatitis A, Typhoid, or Cholera (fever, diarrhea, jaundice) to medical services immediately. If pregnant or planning pregnancy, discuss timing of vaccinations with your GP before travel.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017

https://www.nice.org.uk/guidance/mg2

- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapters 14, 17 and 33; NaTHNaC TravelHealthPro country information pages, current; Summaries of Product Characteristics for the vaccines administered, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Patient Group Direction

for the supply/administration of

Typhoid Vaccine (Typhim Vi)

for protection against

Typhoid (travel)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing

	organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Protection against Typhoid fever for travelers to endemic areas
Inclusion criteria	Adults aged 18 years and over Traveling to areas with high or intermediate prevalence of Typhoid High-risk destinations: South Asia, Southeast Asia, Africa, or Central/South America, confirmed against current NaTHNaC / TravelHealthPro country information Duration of travel: risk increases with longer stays in endemic areas
Exclusion criteria	Known hypersensitivity to vaccine or any excipients Acute illness with fever (defer until recovered) Pregnancy (seek specialist advice)
Cautions	Thrombocytopenia, a bleeding disorder or anticoagulation: not a contraindication. Use a fine needle and apply firm pressure for 2 minutes. Caution in immunocompromised patients (reduced response; seek specialist advice) Vaccinate at least 2 weeks before departure Not protective against paratyphoid A or B Effectiveness: 70-80% protective effect Revaccination recommended every 3 years for continued risk exposure
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Typhim Vi - Typhoid Vi polysaccharide vaccine injection (25 mcg/0.5 mL)
Legal category	POM
Storage	Store at 2-8°C. Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection in the deltoid region
Dose and frequency	Primary course: 0.5 mL single dose Booster: Revaccination required every 3 years if continuing risk
Quantity to be administered and/or supplied	1 × 0.5 mL dose per visit
Maximum or minimum treatment period	Single dose for protection; revaccination every 3 years if required
Adverse effects	Common (>1/100): Injection site reactions (pain, induration, erythema), headache, myalgia Uncommon (>1/1000): Fever, fatigue, nausea, abdominal pain Rare: Lymphadenopathy, allergic reactions Very rare: Guillain-Barré syndrome, anaphylaxis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of administration, dose, form, route and anatomical site, quantity administered, batch number and expiry date • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and the actions taken, and that the vaccine was administered under this PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should

	also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each vaccine. Ensure patient understands the importance of completing the full course if required and understands booster schedules.
Follow-up advice to be given to patient or carer	Complete the full course recommended for the vaccine given: cholera, 2 doses at least 1 week apart, finished at least 1 week before exposure; hepatitis A, a second dose at 6 to 12 months for long-term protection. Attend for booster vaccinations as advised (Hepatitis A at 6-12 months; Typhoid every 3 years; Cholera every 2 years). Report any serious side effects to the GP or seek medical attention immediately. Maintain good food and water hygiene practices during travel (handwashing, avoid raw/undercooked foods, avoid tap water in endemic areas). Obtain comprehensive travel health insurance that covers medical evacuation if traveling to remote areas. Visit your GP or travel health clinic at least 4-6 weeks before departure for full pre-travel assessment. Report symptoms of Hepatitis A, Typhoid, or Cholera (fever, diarrhea, jaundice) to medical services immediately. If pregnant or planning pregnancy, discuss timing of vaccinations with your GP before travel.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapters 14, 17 and 33; NaTHNaC TravelHealthPro country information pages, current; Summaries of Product Characteristics for the vaccines administered, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Patient Group Direction

for the supply/administration of

Cholera Vaccine (Dukoral)

for protection against

Cholera (travel)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal

	• All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Protection against Cholera for travelers to endemic areas with high risk of cholera
Inclusion criteria	Adults aged 18 years and over Traveling to areas with active cholera transmission or at high risk (particularly Africa, South Asia, parts of Latin America) Humanitarian workers, healthcare workers, or those with occupational exposure risk Travelers with planned extended stays in endemic areas with poor sanitation
Exclusion criteria	Known hypersensitivity to the vaccine or any excipient (including formaldehyde) Acute illness with fever or gastrointestinal symptoms (defer until recovered) Pregnancy (seek specialist advice) Severe immunocompromise: current chemotherapy or other immunosuppressive therapy for malignancy; a solid organ or bone marrow transplant within the previous 6 months; or systemic corticosteroids at 20 mg/day prednisolone (or 2 mg/kg/day) or more for 2 weeks or longer within the previous 4 weeks. The vaccine is inactivated and cannot cause infection, but the response is likely to be inadequate: refer for specialist advice rather than supplying.
Cautions	Caution with fever or acute gastrointestinal illness (defer until resolved) Give as set out in the route row: buffer dissolved in about 150 mL of cool water with the whole vial added, and nothing to eat or drink, and no other oral medicines, for 1 hour before and 1 hour after Antibiotics (if recently prescribed for enteric infection) may reduce vaccine effectiveness Other immunosuppression below the exclusion threshold (for example HIV infection, immunosuppressive medicines at a lower dose, or a transplant more than 6 months ago): the vaccine may be given but the response may be reduced. Seek specialist advice where the degree of immunosuppression is uncertain, and advise that food and water precautions remain essential. Vaccine does not prevent cholera due to other strains (limited cross-protection)

	Protection is partial (approximately 85-90% in first year, declining thereafter)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Dukoral - Oral inactivated cholera vaccine containing recombinant B subunit (rCTB) and inactivated whole cells
Legal category	POM
Storage	Store at 2-8°C. Do not freeze. Protect from light. Keep buffer sachet sealed until use.
Route/method of administration	Oral. Dissolve the buffer sachet in about 150 mL of cool water, add the whole 3 mL vial of vaccine suspension, and drink within 2 hours. No food or drink for 1 hour before and 1 hour after; no other oral medicines within 1 hour either side. Complete the course at least 1 week before potential exposure.
Dose and frequency	Primary course: 2 doses, 1-6 weeks apart Booster: Single dose every 2 years if continuing risk in endemic area
Quantity to be administered and/or supplied	2 doses for the primary course (each dose: one 3 mL vial mixed with buffer)
Maximum or minimum treatment period	Primary course requires 2 doses over 1-6 weeks; booster every 2 years if required
Adverse effects	Common (>1/100): Abdominal pain, diarrhea, nausea, vomiting, loss of appetite Uncommon (>1/1000): Headache, fatigue, dizziness, fever, myalgia Rare: Vomiting, constipation, allergic reactions Very rare: Anaphylaxis, severe allergic reactions

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP

	• name of healthcare practitioner • name and brand of medication • date of administration, dose, form, route and anatomical site, quantity administered, batch number and expiry date • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and the actions taken, and that the vaccine was administered under this PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each vaccine. Ensure patient understands the importance of completing the full course if required and understands booster schedules.
Follow-up advice to be given to patient or carer	Complete the full course recommended for the vaccine given: cholera, 2 doses at least 1 week apart, finished at least 1 week before exposure; hepatitis A, a second dose at 6 to 12 months for long-term protection. Attend for booster vaccinations as advised (Hepatitis A at 6-12 months; Typhoid every 3 years; Cholera every 2 years). Report any serious side effects to the GP or seek medical attention immediately. Maintain good food and water hygiene practices during travel (handwashing, avoid raw/undercooked foods, avoid tap water in endemic areas). Obtain comprehensive travel health insurance that covers medical evacuation if traveling to remote areas. Visit your GP or travel health clinic at least 4-6 weeks before departure for full pre-travel assessment. Report symptoms of Hepatitis A, Typhoid, or Cholera (fever, diarrhea, jaundice) to medical services immediately. If pregnant or planning pregnancy, discuss timing of vaccinations with your GP before travel.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapters 14, 17 and 33; NaTHNaC TravelHealthPro country information pages, current; Summaries of Product Characteristics for the vaccines administered, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Records: traceability

RECORD THE VACCINE NAME AND BATCH NUMBER for every dose administered. This is a requirement for any biological product and it is what makes a recall actionable.

Record also the expiry date, the dose, the route, the anatomical site, and where more than one vaccine was given at the same visit, the site of each.

Record the name and registration number of the person administering, and that the vaccine was given under this PGD.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILLICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v008

Issued: 30 September 2026

Supersedes: Version 007, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v007

Issued: 14 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 4: the Dukoral exclusion for severe immunocompromise is now defined (current chemotherapy or other immunosuppressive therapy for malignancy, a transplant within the previous 6 months, or systemic corticosteroids at 20 mg/day prednisolone or 2 mg/kg/day or more for 2 weeks or longer within the previous 4 weeks, the definition used in the dengue PGD), and the Dukoral caution now covers other immunosuppression below that threshold, so the boundary between the exclusion and the caution is stated.

Decision 5: one Agreement to practise page, one premises block, one practitioner table, one adoption block and one change history for the document, after the last arm; the copies that followed the first two arms are removed.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The title and the three covers now say the vaccines are for protection against hepatitis A, typhoid and cholera, not 'for the treatment of Travel Health'.

The typhoid inclusion criterion lists the high-risk regions as destinations, confirmed against NaTHNaC / TravelHealthPro, rather than as 'groups'.

The Dukoral caution about taking it on an empty stomach with water now matches the route row (buffer in about 150 mL of water, nothing by mouth for 1 hour either side).

The follow-up advice under all three vaccines now gives the course completion advice for the vaccine given, instead of the cholera sentence under every vaccine.

The records rows have their punctuation restored and now ask for the anatomical site, batch number and expiry date.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name the Green Book chapters, NaTHNaC and the SmPCs as the governing sources.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027);

narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Observation after vaccination, Cold chain and excursions, Disposal, Records: traceability, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The document would otherwise have expired on 31 October 2026.

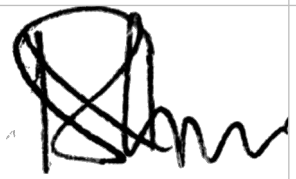
Hepatitis A: Havrix Monodose is 1440 EL.U in a 1.0 mL dose; the document said 0.5 mL in three places, which is half a dose. Corrected to Havrix 1.0 mL, Avaxim 0.5 mL.

Hepatitis A and typhoid: thrombocytopenia was both an exclusion and a caution. Neither SmPC contraindicates it. Removed from exclusions; the caution now states the technique.

Cholera (Dukoral): a dose is a 3 mL vial, not 1.5 mL; gentamicin is not an excipient; the administration instruction now follows SmPC 4.2 and 6.6 (buffer in 150 mL water, whole vial, drink within 2 hours, nothing by mouth for 1 hour either side, complete 1 week before exposure).

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.